

# 15. Management of Diabetes in Pregnancy: Standards of Care in Diabetes—2026

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# DIABETES IN PREGNANCY

The prevalence of diabetes in pregnancy has been increasing in the U.S. in parallel with the worldwide epidemic of obesity. Not only is the prevalence of type 1 diabetes and type 2 diabetes increasing in individuals of reproductive age but there is also a dramatic increase in the reported rates of gestational diabetes mellitus (GDM). Diabetes confers significantly greater maternal and fetal risk that is largely related to the degree of hyperglycemia but also is related to chronic complications and comorbidities of diabetes. In general, specific risks of diabetes in pregnancy include spontaneous abortion, fetal anomalies, preeclampsia, fetal demise, macrosomia, neonatal hypoglycemia, neonatal hyperbilirubinemia, and neonatal respiratory distress syndrome. In addition, exposure to hyperglycemia in utero increases the risks of obesity, hypertension, and type 2 diabetes in offspring later in life (1,2).

# Preconception Counseling Recommendations

1. Starting at puberty and continuing in all people with diabetes and childbearing potential, preconception counseling should be incorporated into routine diabetes care. A 2. Family planning should be discussed, and effective contraception (with consideration of long-acting, reversible contraception) should be prescribed and used until an individual’s treatment plan and A1C are optimized for pregnancy. A 3. Preconception counseling should address the importance of achieving glucose levels as close to normal as is safely possible without excessive hypoglycemia, ideally A1C  $\leq 6.5\%$  ( $\leq 48$  mmol/mol), to reduce the risk of congenital anomalies, preeclampsia, macrosomia, preterm birth, and other complications. A

\*A complete list of members of the American Diabetes Association Professional Practice Committee for Diabetes can be found at <https://doi.org/10.2337/dc26-SINT>. Duality of interest information for each contributor is available at <https://doi.org/10.2337/dc26-SDIS>.

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# Management of Diabetes in Pregnancy

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# 15.4

Individuals with a history of gestational diabetes mellitus (GDM) should seek preconception screening for diabetes and preconception care to identify and treat hyperglycemia and prevent congenital malformations and other adverse maternal and fetal outcomes. In a systematic review and meta-analysis of observational studies, preconception care for pregnant individuals with preexisting diabetes was associated with lower A1C and reduced risks of congenital anomalies (given that organogenesis occurs primarily at 5–8 weeks of gestation), preeclampsia, and preterm birth (13–17).

Weight should be assessed at the preconception evaluation. Counseling on the specific risks of obesity in pregnancy and lifestyle interventions to prevent and treat obesity, including referral to a registered dietitian nutritionist (RDN), is recommended regardless of diabetes status (24). In a randomized trial of individuals with overweight or obesity and a history of GDM, weight loss prior to a subsequent pregnancy was associated with a lower risk of GDM recurrence, especially when weight loss was  $\geq 5\%$  (odds ratio [OR] 0.18, 95% CI 0.04–0.88) (25).

Preconception counseling for pregnant people with preexisting type 1 or type 2 diabetes is highly effective in reducing the risk of congenital malformations and decreasing the risk of preterm delivery and admission to neonatal intensive care units (NICU). Preconception counseling is also associated with reductions in perinatal mortality and small-for-gestational-age birth weight (3). A key point is the need to incorporate a question about plans for pregnancy into the routine primary and gynecologic care of people with diabetes.

There are opportunities at any health care visit to educate all adults and adolescents with diabetes and childbearing potential about the risks of unplanned pregnancies and about improved maternal and fetal outcomes with pregnancy planning (4). Education and counseling should be offered, even when individuals already use contraception or do not intend to conceive (5). Effective preconception counseling could avert substantial health and associated cost burdens related to the offspring (6).

Family planning should be discussed, including the benefits of long-acting, reversible contraception, and effective contraception should be prescribed and used until the individual is prepared and ready to become pregnant (7–12). All individuals with diabetes and childbearing potential should be informed about the importance of achieving and maintaining as near euglycemia as safely possible prior to conception and throughout pregnancy.

#### # Preconception Care Recommendations

##### # 15.5

Individuals with preexisting diabetes who are planning a pregnancy should ideally begin receiving interprofessional care prior to conception, which includes an endocrinology health care professional, maternal-fetal medicine specialist, registered dietitian nutritionist, and diabetes care and education specialist, when available. B

##### # 15.6

In addition to focused attention on achieving glycemic goals, a standard preconception care should be augmented with extra focus on nutrition, physical activity, diabetes self-care education, and screening for diabetes comorbidities and complications. B

##### # 15.7

Individuals with preexisting diabetes who are planning a pregnancy or who have become pregnant should be counseled on the risk of development of diabetic embryopathy, especially anencephaly, microcephaly, congenital heart disease, kidney anomalies, and caudal regression, directly proportional to elevations in A1C during the first 10 weeks of pregnancy (13).

Although observational studies are confounded by the association between elevated periconceptional A1C and other engagement in self-care behaviors, the quantity and consistency of data are convincing and support the recommendation to optimize glycemia prior to conception with an A1C  $\leq 6.5\%$  ( $\leq 48$  mmol/mol), as this is associated with the lowest risk of complications.

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and/or progression of diabetic retinopathy. Dilated eye examinations should occur ideally before pregnancy as well as in the first trimester, and then pregnant individuals should be monitored every trimester and for 1 year postpartum as indicated by the degree of retinopathy and as recommended by the eye care health care professional.

# Table 15.1—Checklist for preconception care for people with prediabetes, diabetes, or a history of gestational diabetes mellitus

Preconception education should include:

- Comprehensive nutrition assessment and recommendations for: - Overweight and obesity or underweight - Meal planning - Correction of nutritional deficiencies - Caffeine intake - Safe food preparation technique - Lifestyle recommendations for: - Regular moderate exercise - Avoidance of hyperthermia (hot tubs) - Adequate sleep - Comprehensive diabetes self-management education - Counseling on diabetes in pregnancy per current standards, including natural history of insulin resistance in pregnancy and postpartum; preconception glycemic goals; avoidance of DKA or severe hyperglycemia; avoidance of severe hypoglycemia; progression of retinopathy in individuals with preexisting diabetes; PCOS (if applicable); fertility in people with diabetes; genetics of diabetes; risks to pregnancy including miscarriage, stillbirth, congenital malformations, macrosomia, preterm labor and delivery, hypertensive disorders in pregnancy - Supplementation - Folic acid supplement (400–800 µg/day routine) - Appropriate use of over-the-counter medications and supplements

Health assessment and plan should include:

- General evaluation of overall health - Evaluation of diabetes and its comorbidities and complications, including DKA or severe hyperglycemia; severe hypoglycemia/hypoglycemia unawareness; barriers to care; comorbidities such as hyperlipidemia, hypertension, MASLD, PCOS, and thyroid dysfunction; complications such as macrovascular disease in individuals with preexisting diabetes, nephropathy, neuropathy (including autonomic bowel and bladder dysfunction), and retinopathy - Evaluation of obstetric or gynecologic history, including a history of cesarean section, congenital malformations or fetal loss, current methods of contraception, hypertensive disorders of pregnancy, postpartum hemorrhage, preterm delivery, previous macrosomia, Rh incompatibility, and thrombotic events (DVT/PE) - Review of current medications and appropriateness during pregnancy

Screening should include:

- Diabetes complications and comorbidities, including comprehensive foot exam; comprehensive ophthalmologic exam; ECG in individuals starting at age 35 years who have cardiac signs or symptoms or risk factors and, if abnormal, further evaluation; lipid panel; serum creatinine; TSH; and urine albumin-to-creatinine ratio - Anemia - Genetic carrier status (based on history): - Cystic fibrosis - Sickle cell anemia - Tay-Sachs disease - Thalassemia - Others if indicated - Infectious disease (per ACOG guidelines)

Preconception plan should include:

- Immunizations (per ACOG guidelines) (203–205) - Nutrition and medication plan to achieve glycemic goals prior to conception, including appropriate implementation of blood glucose monitoring, continuous glucose monitoring (if indicated and appropriate), and pump technology (if indicated and appropriate) - Contraceptive plan to prevent pregnancy until glycemic goals are achieved - Management plan for general health, gynecologic concerns, comorbid conditions, or complications, if present, including hypertension, nephropathy, retinopathy; Rh incompatibility; and thyroid dysfunction

The most important diabetes-specific component of preconception care is the attainment of glycemic goals prior to conception. Diabetes-specific counseling should include an explanation of the risks to mother and fetus related to pregnancies.

Created using information from American College of Obstetricians and Gynecologists (ACOG) (5) and others (28,30). DKA, diabetic ketoacidosis; DVT/PE, deep vein thrombosis/pulmonary embolism; ECG, electrocardiogram; MASLD, metabolic dysfunction-associated steatotic liver disease; PCOS, polycystic ovary syndrome; TSH, thyroid-stimulating hormone.

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associated with diabetes and the ways to reduce risks, including glycemic goal setting, lifestyle and behavioral management, and medical nutrition therapy (3). Prior to conception, it is recommended to achieve glucose levels as close to normal as is safely possible without excessive hypoglycemia, ideally A1C  $\leq 6.5\%$  ( $\leq 48$  mmol/mol), and preconception glycemic goals should be achieved using therapies approved for use in pregnancy. Setting preconception glucose goals is helpful in guiding insulin dose adjustments. The following preconception glucose goals have been advised: preprandial glucose 80–110 mg/dL (4.4–6.1 mmol/L) and 2 h postprandial glucose  $\leq 155$  mg/dL ( $\leq 8.6$  mmol/L) (31). Prandial insulin dose should be reduced for postprandial glucose  $\leq 100$  mg/dL (5.6 mmol/L).

For individuals with preexisting diabetes, the presence of microvascular complications is associated with higher risk of disease progression and adverse pregnancy outcomes (32). Diabetes-specific testing should include A1C, creatinine, and urinary albumin-to-creatinine ratio. Special attention should be paid to the review of the medication list for potentially harmful drugs, e.g., ACE inhibitors (33), angiotensin receptor blockers (33), and lipid-lowering medications including statins in some cases (34). For individuals using medications that are not approved for use in pregnancy (such as some glucose-lowering, lipid-lowering, and antihypertensive agents), preconception care should include recommendations for when changes in medications should occur to stabilize the conditions and risk factors managed by these medications (such as glucose levels, weight, lipids, and blood pressure) on alternate therapies prior to pregnancy. Glucagon-like peptide 1 receptor agonists (GLP-1 RAs) and dual glucose-dependent insulinotropic polypeptide (GIP) and GLP-1 RAs should be discontinued prior to pregnancy, and contraception should be used while taking them (see section 9, “Pharmacologic Approaches to Glycemic Treatment”). The manufacturer prescribing information recommends discontinuation of semaglutide at least 2 months before a planned pregnancy due to its long half-life (35). No recommendation is included in the U.S. prescribing information for tirzepatide, although the Canadian manufacturer prescribing information recommends tirzepatide discontinuation at least 1 month before pregnancy (36). Importantly, before attempting conception, preconception glycemic goals should be achieved following discontinuation of GLP-1 RA therapy or dual GIP and GLP-1 RA therapy, with initiation and titration of insulin therapy when needed to achieve preconception glycemic goals.

### # Insulin Physiology

Pregnancy in people with normal glucose metabolism is characterized by fasting glucose levels that are lower than those in the nonpregnant state due to insulin-independent glucose uptake by the fetus and placenta and by mild postprandial hyperglycemia and carbohydrate intolerance as a result of diabetogenic placental factors. Early pregnancy may be a time of enhanced insulin sensitivity and lower glucose levels and is followed by progressive insulin resistance in the second and third trimesters (38–40). Insulin resistance drops rapidly with the delivery of the placenta. In people with normal pancreatic function, insulin production is sufficient to meet the challenge of this physiological insulin resistance and to maintain normal glucose levels. However, in people with diabetes, hyperglycemia occurs if treatment is not adjusted appropriately.

### # Recommendations

15.8 Fasting, preprandial, and postprandial blood glucose monitoring are recommended in individuals with diabetes in pregnancy to achieve optimal glucose levels. Glucose goals are fasting plasma glucose  $\leq 95$  mg/dL ( $\leq 5.3$  mmol/L) and either 1-h postprandial glucose  $\leq 140$  mg/dL ( $\leq 7.8$  mmol/L) or 2-h postprandial glucose  $\leq 120$  mg/dL ( $\leq 6.7$  mmol/L). B

15.9 Due to increased red blood cell turnover, A1C is slightly lower during pregnancy in people with and without diabetes. Ideally, the A1C goal in pregnancy is  $\leq 6\%$  ( $\leq 42$  mmol/mol) if this can be achieved without significant hypoglycemia, but the goal may be relaxed to  $\leq 7\%$  ( $\leq 53$  mmol/mol) if necessary to prevent hypoglycemia. B

15.10 Continuous glucose monitoring (CGM) can help to achieve glycemic goals (e.g., time in range, time above range) A and A1C goal B in type 1 diabetes and pregnancy and may be beneficial for other types of diabetes in pregnancy. E

15.11 Recommend CGM to pregnant individuals with type 1 diabetes. A In conjunction with aims to achieve traditional pre- and postprandial glycemic goals, CGM can reduce the risk for large-for-gestational-age infants and neonatal hypoglycemia in pregnancy complicated by type 1 diabetes. A

| Table 15.2: Recommended Goals for Pregnant People with Type 1 or Type 2 Diabetes | |

| Parameter                | Goal                     | Fasting |
|--------------------------|--------------------------|---------|
| Plasma Glucose           | <95 mg/dL (<5.3 mmol/L)  |         |
| 1-h Postprandial Glucose | <140 mg/dL (<7.8 mmol/L) |         |
| 2-h Postprandial Glucose | <120 mg/dL (<6.7 mmol/L) |         |

Lower limits are based on the mean of normal blood glucose in pregnancy (43) but do not apply to individuals with type 2 diabetes treated with nutrition alone. Hypoglycemia in pregnancy is as defined and discussed in Recommendations 6.10–6.19 (see section 6, “Glycemic”).

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# Table 15.2—Blood glucose goals in pregnancies associated with diabetes

| Glucose measurement      | Blood glucose goal              | Type 1 diabetes or type 2 diabetes1 | GDM treated with insulin  | GDM not treated with insulin |
|--------------------------|---------------------------------|-------------------------------------|---------------------------|------------------------------|
| Fasting glucose          | 70–95 mg/dL (3.9–5.3 mmol/L)    | 70–95 mg/dL (3.9–5.3 mmol/L)        | <95 mg/dL (<5.3 mmol/L)   | <95 mg/dL (<5.3 mmol/L)      |
| 1-h postprandial glucose | 110–140 mg/dL* (6.1–7.8 mmol/L) | 110–140 mg/dL* (6.1–7.8 mmol/L)     | <140 mg/dL* (<7.8 mmol/L) | <140 mg/dL* (<7.8 mmol/L)    |
| 2-h postprandial glucose | 100–120 mg/dL (5.6–6.7 mmol/L)  | 100–120 mg/dL (5.6–6.7 mmol/L)      | <120 mg/dL (<6.7 mmol/L)  | <120 mg/dL (<6.7 mmol/L)     |

Gestational diabetes mellitus (GDM) blood glucose goals shown are recommended by the Fifth International Workshop-Conference on Gestational Diabetes Mellitus<sup>2</sup>. Lower glucose limits do not apply to individuals with type 2 diabetes treated with nutrition alone. Aim for less stringent goals if these cannot be achieved without significant hypoglycemia, based on clinical experience and individualization of care.

\*Optimal goal includes either a 1-h postprandial glucose level or 2-h postprandial glucose level within column of type of diabetes.

Goals, Hypoglycemia, and Hyperglycemic Crises. The most appropriate hypoglycemia threshold level in pregnancy has not been validated but has ranged from 60 to 70 mg/dL (3.3 to 3.9 mmol/L) in the past. Current recommendations for hypoglycemia thresholds include blood glucose 70 mg/dL (3.9 mmol/L) and sensor glucose 63 mg/dL (3.5 mmol/L)<sup>3,4</sup>. These fasting or premeal and postprandial glucose values represent optimal levels if they can be achieved safely. In practice, it may be challenging for a person with type 1 diabetes to achieve these goals without hypoglycemia, particularly those with a history of recurrent hypoglycemia or impaired awareness of hypoglycemia. If an individual cannot achieve these goals without significant hypoglycemia, aim for less stringent goals based on clinical experience and individualization of care.

For individuals with GDM, the glucose goals recommended by the Fifth International Workshop-Conference on Gestational Diabetes Mellitus should be used<sup>2</sup> (Table 15.2). For individuals found to have hyperglycemia less than overt diabetes on early pregnancy screening, the benefits of treatment remain uncertain, controversy exists regarding optimal management strategies, and professional society guidance is conflicting<sup>5,6</sup>. While hyperglycemia less than overt diabetes in early pregnancy has been associated with increased risk of adverse pregnancy and neonatal outcomes and higher risk of a later GDM diagnosis (see Recommendation 2.32b)<sup>7–11</sup>, it has not been established that universal treatment in early pregnancy (prior to the usual time of GDM diagnosis and treatment at 24–28 weeks’ gestation) is beneficial for all individuals in this group.

Most randomized controlled trials (RCTs) of treatment of early abnormal glucose have been underpowered for outcomes. One large multicenter RCT (Treatment of Booking Gestational Diabetes Mellitus [TOBOGM] trial), which enrolled pregnant individuals with at least one risk factor for hyperglycemia, administered early screening (mean 15.6 ± 2.5 weeks) for GDM with a 75-g OGTT<sup>12</sup>. Individuals who met World Health Organization criteria for GDM were randomized to receive early treatment or a repeat OGTT at 24–28 weeks (with deferred

treatment if indicated). The first primary outcome measure was an adverse neonatal composite outcome including birth <37 weeks, birth weight  $\geq 4.5$  kg, birth trauma, neonatal respiratory distress within 24 h of birth, phototherapy, stillbirth neonatal death, or shoulder dystocia. Early GDM treatment resulted in a modest improvement in the composite adverse neonatal outcome (24.9% early treatment vs. 30.5% control group, relative risk 0.82 [0.68–0.98]), although this was driven primarily by differences in rates of neonatal respiratory distress between groups that included neonates requiring  $\geq 4$  h of supplemental oxygen who may not have required a higher level of respiratory care. In prespecified subgroup analyses, there was a suggestion of more benefit among individuals who had the OGTT at <14 weeks and among those with OGTT glycemic values in higher glycemic ranges<sup>12</sup>.

Subsequent secondary analyses have shed additional light on which subgroups may derive greatest benefit from treatment. Importantly, almost one-third of individuals randomized to observation demonstrated regression of GDM when retested at 24–28 weeks' gestation. Pregnancy outcomes among participants with regression were similar to those of individuals without GDM<sup>13</sup>, and these individuals are therefore unlikely to benefit from treatment in this subgroup. While RCT data to guide clinical practice in this subgroup are lacking, it is possible that selective treatment of early pregnancy hyperglycemia within this subgroup would lead to more pronounced reductions in adverse pregnancy and neonatal outcomes.

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Therefore, the benefits of treatment of early abnormal glucose metabolism remain uncertain. Nutrition counseling and periodic testing of fasting glucose levels (e.g., 3–4 times per week) are suggested. Testing frequency may proceed to daily, and treatment may be intensified, if fasting plasma glucose is predominantly  $\geq 110$  mg/dL ( $\geq 6.1$  mmol/L) prior to 15 weeks of gestation. Individualized discussion of areas of uncertainty and the option to treat for fasting glucose  $\geq 110$  mg/dL ( $\geq 6.1$  mmol/L) via shared decision-making is reasonable. When treatment for early abnormal glucose metabolism is initiated, fasting and postprandial glucose levels should be monitored and the standard glucose goals recommended in pregnancies associated with diabetes (Table 15.2) should be used.

# A1C in Pregnancy

In studies of individuals without preexisting diabetes, increasing A1C levels within the normal range are associated with adverse outcomes (59). In the Hyperglycemia and Adverse Pregnancy Outcome (HAPO) study, increasing levels of glycemia were also associated with worsening outcomes (21). Observational studies in preexisting diabetes and pregnancy show the lowest rates of adverse fetal outcomes in association with A1C <6–6.5% (<42–48 mmol/mol) early in gestation (14,15,17,60). Clinical trials have not evaluated the risks and benefits of achieving these goals, and treatment goals should account for the risk of maternal hypoglycemia in setting an individualized goal of <6% (<42 mmol/mol) to <7% (<53 mmol/mol). Due to physiological increases in red blood cell turnover, A1C levels fall during normal pregnancy (61,62). Additionally, as A1C represents an integrated measure of glucose, it may not fully capture postprandial hyperglycemia, which drives macrosomia. Thus, although A1C may be useful, it should be used as a secondary measure of glycemia in pregnancy, after glucose monitoring.

In the second and third trimesters, A1C <6% (<42 mmol/mol) has the lowest risk of large-for-gestational-age infants (60,63,64), preterm delivery (65), and preeclampsia (1,66). Taking all of this into account, a goal of <6% (<42 mmol/mol) is optimal during pregnancy if it can be achieved without significant hypoglycemia, which, in addition to the usual adverse sequelae, may increase the risk of low birth weight (67,68).

# Continuous Glucose Monitoring in Pregnancy

The Continuous Glucose Monitoring in Pregnant Women With Type 1 Diabetes Trial (CONCEPTT) was an RCT of CGM in addition to standard care, including optimization of pre- and postprandial glucose goals, versus standard care for pregnant people with type 1 diabetes. It demonstrated the value of using real-time CGM in

pregnant individuals with type 1 diabetes by showing a mild improvement in A1C and significant improvements in the maternal glucose time in range (TIR) and time above range (TAR), without an increase in hypoglycemia, and reductions in large-for-gestational-age births, length of infant hospital stays, and severe neonatal hypoglycemia (69).

An observational cohort study that evaluated the glycemic variables reported using CGM systems found that lower mean glucose, lower SD, and higher percentage of TIR were associated with lower risks of large-for-gestational-age births and other adverse neonatal outcomes (70). Data from one study suggest that use of the CGM-reported mean glucose is superior to use of estimated A1C, glucose management indicator, and other calculations to estimate A1C, given the changes to A1C that occur in pregnancy (71).

One RCT and several observational studies have found that a 5% increase in CGM TIR was associated with improvements in neonatal morbidity, including large-for-gestational-age births and neonatal intensive care unit admissions (69,70,72,73). CGM TIR can be used for assessment of glycemic outcomes in people with type 1 diabetes, but it does not provide actionable data to address fasting and postprandial hypoglycemia or hyperglycemia. The cost of CGM use by pregnant individuals with type 1 diabetes is offset by improved maternal and neonatal outcomes (74).

At this time, there are insufficient data to support the use of CGM in all people with type 2 diabetes or GDM (75–79). One RCT evaluating use of CGM or standard BGM in GDM found no significant difference in the primary end point, a composite of adverse perinatal and neonatal outcomes (79). TIR was lower in the CGM group, though CGM use was preferred by participants. In contrast, another RCT evaluating use of CGM or standard BGM in GDM found no significant difference in the primary end point, a composite of adverse perinatal and neonatal outcomes (79). Goals for sensor glucose ranges in pregnancy are:

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- Goal sensor glucose range 63–140 mg/dL may not be sufficient to achieve glycemic goals. B Other oral and noninsulin injectable glucose-lowering medications lack long-term safety data and are not recommended. E

- (3.5–7.8 mmol/L): TIR, goal >70% Time below range (TBR) (<3.5 mg/dL [ $\leq$ 3.5 mmol/L]): level 1 TBR, goal <4% TBR (<54 mg/dL [ $\leq$ 3.0 mmol/L]): level 2 TBR, goal <1% TAR (>140 mg/dL [ $>$ 7.8 mmol/L]): TAR, goal <25%

Goals for time spent in each range are specific for pregnant individuals with type 1 diabetes.

## # MANAGEMENT OF DIABETES IN PREGNANCY

### # Recommendations

15.14 Nutrition counseling before and during pregnancy should promote an eating pattern including fruits, vegetables, legumes, whole grains, nuts, seeds, fish, and other lean protein, which will provide a balance of macronutrients and healthy n-3 fatty acids. C

15.15 Lifestyle behavior change is an essential component of management of GDM and may suffice as treatment for many individuals. Insulin should be added if needed to achieve glycemic goals. A

15.16 Telehealth visits used in combination with in-person visits for pregnant people with GDM can improve outcomes compared with standard in-person care alone. A

15.17 Insulin should be used to manage type 1 diabetes in pregnancy A and is the preferred agent for the management of GDM A and type 2 diabetes in pregnancy. B

15.18 Either multiple daily injections or insulin pump technology can be used in pregnancy complicated by type 1 diabetes. C

15.19 Automated insulin delivery (AID) systems with pregnancy-specific glucose targets are recommended for pregnant individuals with type 1 diabetes. A

15.20 AID systems without pregnancy-specific glucose targets or a pregnancy-specific algorithm may be considered for select pregnant individuals with type 1 diabetes when used with assistive techniques and working with experienced health care teams. B

15.21 Metformin and glyburide, individually or in combination, should not be used as first-line agents for management of diabetes in pregnancy, as both cross the placenta to the fetus A and.

#### # Medical Nutrition Therapy

The management of pregnancies associated with diabetes includes appropriate nutrition, lifestyle and behavior management, physical activity goals, and pharmacotherapy to support the maternal, fetal, and placental needs and reach glycemic goals regardless of the diabetes type.

In people with preexisting diabetes, glycemic goals are usually achieved through a combination of insulin administration and medical nutrition therapy. Because glycemic goals in pregnant individuals are stricter than in nonpregnant individuals, it is important that pregnant people with diabetes consume consistent amounts of carbohydrates to match their prandial insulin dosages or use insulin-to-carbohydrate ratios to match prandial insulin dosages to the carbohydrates being consumed in order to minimize postprandial hyperglycemia or hypoglycemia.

Referral to an RDN is important to establish a food plan and to determine weight gain goals. The quality of the carbohydrates should be evaluated. A subgroup analysis of the CONCEPTT study demonstrated that the food choices of individuals planning pregnancy and currently pregnant assessed during the run-in phase prior to randomization were characterized by high-fat, low-fiber, and poor-quality carbohydrate intakes. Fruit and vegetable consumption was inadequate, with one in four participants at risk for micronutrient deficiencies, highlighting the importance of medical nutrition therapy (82).

An expert panel on nutrition in pregnancy and the U.S. Department of Health and Human Services recommend a balance of macronutrients. An eating pattern that severely restricts any macronutrient class should be avoided, specifically the ketogenic eating pattern that lacks carbohydrates, the paleo eating pattern because of dairy restriction, and any eating pattern characterized by excess saturated fats (83).

#### # Lifestyle and Behavioral Management

Although there is some heterogeneity, many RCTs and a Cochrane review suggest that the risk of GDM may be reduced by appropriate nutrition, exercise, and lifestyle changes.

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Lifestyle counseling, particularly when interventions are started during the first trimester or early in the second trimester (91–93). After diagnosis of GDM, treatment starts with medical nutrition therapy, physical activity, and weight management, depending on pregestational weight, as outlined in this section. Depending on the population, studies suggest that 70–85% of people diagnosed with GDM under Carpenter-Coustan criteria can manage GDM with lifestyle modification alone; it is anticipated that this proportion will be even higher if the lower International Association of the Diabetes and Pregnancy Study Groups (94) diagnostic thresholds are used.

#### # Pharmacologic Therapy

##### # Insulin

Insulin should be used to manage type 1 diabetes in pregnancy and is preferred for the management of type 2 diabetes in pregnancy and GDM. The physiology of pregnancy necessitates frequent titration of insulin to match changing requirements and underscores the importance of daily and frequent glucose monitoring. In early pregnancy, many people with type 1 diabetes will have lower insulin requirements and an increased risk for hypoglycemia (38). At around 16 weeks, insulin resistance begins to increase, and total daily insulin doses increase linearly by ~5% per week through week 36. This usually results in a doubling of daily insulin dose compared with the prepregnancy requirement. While there is an increase in both basal and bolus insulin

requirements, bolus insulin requirements take up a larger proportion of overall total daily insulin needs in individuals with preexisting diabetes as pregnancy progresses (39,40). The insulin requirement levels off toward the end of the third trimester. A rapid and significant reduction in insulin requirements may indicate the development of placental insufficiency (100), although data are conflicting (101).

Optimal glycemic goals are often easier to achieve during pregnancy with type 2 diabetes than with type 1 diabetes but can require much higher doses of insulin, sometimes necessitating concentrated insulin formulations. It is recommended that insulin management be performed with interprofessional team members with relevant expertise.

#### # Health Care Delivery for People With Diabetes in Pregnancy

As discussed in PRECONCEPTION CARE, team-based care is recommended either through a single specialized center (when available) or multiple centers with interprofessional team members as part of the care plan. None of the currently available human insulin preparations have been demonstrated to cross the placenta (102–107). Insulins studied in RCTs are preferred (108–110) over those studied in cohort studies (111), which are preferred over those studied in case reports only. Basal insulin options in pregnancy include insulin glargine (long-acting), NPH insulin (intermediate-acting), and insulin degludec (ultralong-acting). Insulin detemir (the only basal insulin with RCT data supporting its use during pregnancy) was removed from the market by the manufacturer and is no longer available. Observational studies of glargine use during pregnancy have not shown evidence of increased adverse fetal outcomes with use of insulin glargine versus NPH insulin (111), and insulin glargine is widely used during pregnancy.

Automated insulin delivery (AID) systems have been studied in pregnancy and postpartum. The use of AID systems in pregnancy affected by diabetes presents particular challenges, as the current U.S. Food and Drug Administration (FDA)-approved AID systems (except for one that has been FDA approved but is not yet commercially available in the U.S.) do not have algorithms designed to achieve pregnancy-specific glucose goals.

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and the lowest glucose target available &#x3C;63 mg/dL (&#x3C;3.5 mmol/L) decreased episodes. One prespecified observational analysis of individuals who continued AID intrapartum and/or early postpartum found improved TIR intrapartum and similar TIR early postpartum in the AID group compared with the standard insulin therapy group, without severe hypoglycemia (126). In a prespecified extension to the AiDAPT trial, in which participants continued either AID or standard insulin therapy for 6 months postpartum, TIR was higher and mean glucose lower in the AID group, without significant differences in hypoglycemia (127). Suggested strategies for postpartum adjustment of AID pump settings have been described (128).

Treatment of GDM with lifestyle and insulin has been demonstrated to improve perinatal outcomes in two large RCTs, as summarized in a U.S. Preventive Services Task Force review (129). Insulin is the first-line agent recommended for the treatment of GDM in the U.S. While individual studies support limited efficacy of metformin (130,131) and glyburide (132) in reducing glucose levels for the treatment of GDM, these agents are not recommended as the first-line treatment of GDM because they are known to cross the placenta and data on long-term safety for offspring is of some concern (20). Furthermore, in separate RCTs, glyburide and metformin failed to achieve adequate glycemic outcomes in 23% and 25–28% of participants with GDM, respectively (133,134).

#### # Sulfonylureas

Sulfonylureas are known to cross the placenta and have been associated with increased neonatal hypoglycemia. Concentrations of glyburide in umbilical cord plasma are approximately 50–70% of maternal levels (133,134). In systematic reviews and meta-analyses, compared with insulin or metformin, glyburide was associated with a higher rate of neonatal hypoglycemia and macrosomia and an increased neonatal abdominal circumference (135,136).

Glyburide was not found to be noninferior to insulin based on a composite outcome of neonatal hypoglycemia, macrosomia, and hyperbilirubinemia among individuals with GDM (137). In a randomized noninferiority trial evaluating insulin compared with an oral glucose-lowering medication strategy (metformin with addition of glyburide and then insulin substitution for glyburide if glycemic goals were not met) in GDM, the oral glucose-lowering medication strategy failed to meet criteria for noninferiority compared with insulin in...

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Preventing large-for-gestational-age infants demonstrated higher BMI and increased obesity in the offspring exposed to metformin. A follow-up study at 5–10 years showed that the offspring had higher BMI, weight-to-height ratios, and waist circumferences and a borderline increase in fat mass. A meta-analysis demonstrated that metformin exposure resulted in smaller neonates with an acceleration of postnatal growth, resulting in higher BMI in childhood. Follow-up of offspring from the Metformin in Women with Type 2 Diabetes in Pregnancy (MiTy Kids) trial showed no differences in anthropometrics of children at 24 months. When prepregnancy metformin use is continued in early pregnancy, it can be discontinued at the end of the first trimester (after organogenesis) to avoid sudden deterioration of glycemia and an associated increased risk of malformations due to hyperglycemia. Continuation of metformin in early pregnancy is considered safe, as first-trimester metformin use has not been associated with increased risk of major congenital malformations, and there is minimal maternal-to-fetal transfer of metformin in early gestation.

There are some people with GDM requiring medical therapy who may not be able to use insulin safely or effectively during pregnancy due to cost, comprehension, or cultural influences. Oral agents may be an alternative for these individuals after discussing the known risks and the need for more long-term safety data in offspring. However, due to the potential for growth restriction or acidosis in the setting of placental insufficiency, metformin should not be used in pregnant people with hypertension or preeclampsia or those at risk for intrauterine growth restriction.

# Special Considerations for Management of Pregnancies With Diabetes

# Recommendation

15.23 Pregnant individuals with type 1 or type 2 diabetes should be prescribed low-dose aspirin 100–150 mg/day starting at 12–16 weeks of gestation to lower the risk of preeclampsia. A dosage of 162 mg/day may be acceptable; currently, in the U.S., low-dose aspirin is available in 81-mg tablets.

Diabetes in pregnancy is associated with an increased risk of preeclampsia. The U.S. Preventive Services Task Force recommends that blood pressure measurements be obtained throughout gestation to screen for hypertensive disorders of pregnancy.

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(166). The Task Force also recommends using low-dose aspirin (81 mg/day) as a preventive medication at 12 weeks of gestation in individuals at high risk for preeclampsia, such as those with type 1 or type 2 diabetes (167). However, a meta-analysis and an additional trial demonstrate that low-dose aspirin <100 mg is not effective in reducing preeclampsia, so a dose of >100 mg is required (168–170). A cost-benefit analysis has concluded that this approach would reduce morbidity, save lives, and lower health care costs (171). There are insufficient data about whether the use of aspirin specifically in pregnant people with preexisting diabetes ultimately reduces the incidence of preeclampsia (172,173), although a meta-analysis showed that preeclampsia reductions occurred with aspirin administration in high-risk groups overall (165). Individuals with GDM may be candidates for aspirin therapy for preeclampsia prevention if they have a single high-risk factor, such as chronic hypertension or an autoimmune disease, or multiple moderate risk factors, such as being nulliparous, having obesity, being age ≥35 years, or other factors per the U.S. Preventive Services Task Force (167). More studies are needed to assess the long-term effects of prenatal aspirin exposure on offspring (172).

## # PREGNANCY AND NON-GLUCOSE-LOWERING DRUG CONSIDERATIONS

### # Recommendations

15.24 In pregnant individuals with diabetes and chronic hypertension, a blood pressure threshold  $\leq 140/90$  mmHg is recommended for initiation and titration of therapy for better pregnancy outcomes. A Therapy should be deintensified if blood pressure is  $\leq 90/60$  mmHg. E

15.25a Potentially harmful medications in pregnancy (e.g., ACE inhibitors, angiotensin receptor blockers, mineralocorticoid receptor antagonists) should be stopped prior to conception and avoided in sexually active individuals of childbearing potential who are not using reliable contraception. B

15.25b In most circumstances, lipid-lowering medications should be stopped prior to conception and avoided in sexually active individuals of childbearing potential with diabetes who are not using reliable contraception. B In some circumstances (familial

tes and chronic hypertension to determine the effect of a lower blood pressure goal ( $\leq 130/80$  mmHg) on pregnancy and neonatal outcomes (175). Participants with lower blood pressure were more likely to be on antihypertensive medications at the start of pregnancy and more likely to have newly diagnosed diabetes prior to 20 weeks. Average blood pressure attained was systolic  $122.0 \pm 5.7$  mmHg vs.  $134.9 \pm 7.4$  mmHg and diastolic  $74.4 \pm 4.0$  mmHg vs.  $81.7 \pm 5.7$  mmHg in the  $\leq 130/80$  mmHg vs.  $130\text{--}139/80\text{--}89$  mmHg groups, respectively. Participants with an average blood pressure  $\leq 130/80$  mmHg were less likely to have the primary composite adverse perinatal outcome (19.3% vs. 46.5%, adjusted relative risk 0.43, 95% CI 0.30–0.61) and its components: preeclampsia with severe features (14.0 vs. 40.5%, adjusted risk ratio 0.35, 95% CI 0.23–0.54) and indicated preterm birth  $\leq 35$  weeks (8.0% vs. 20.1%, adjusted risk ratio 0.44, 95% CI 0.24–0.79),  $P \leq 0.01$  for both. They were less likely to have adverse perinatal outcomes: birth weight  $\leq 10$ th percentile (3.3% vs. 8.8%, adjusted risk ratio 0.37, 95% CI 0.14–0.96,

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discussion of risks and benefits in pregnant individuals at high risk, such as those with a history of atherosclerotic cardiovascular disease or familial hypercholesterolemia (homozygous or severe heterozygous) (34). Hydrophilic statins, such as pravastatin, may be associated with less fetal harm than lipophilic statins (180). Pravastatin has been studied in multiple pregnancy trials administering therapy at various time points in gestation with the aim to reduce preeclampsia risk, and although its ability to do so is inconclusive to date, there does not appear to be increased neonatal mortality or morbidity associated with its use during gestation (34). In pregnant individuals with a history of severe hypertriglyceridemia, triglyceride levels should be monitored, and nutrition interventions with or without pharmacologic therapy for severe hypertriglyceridemia should be instituted when indicated to reduce pancreatitis risk (34).

### # Diabetes Treatment Postpartum

For individuals requiring insulin after delivery, insulin sensitivity increases dramatically with the delivery of the placenta. In one study, insulin requirements in the immediate postpartum period were roughly 34% lower than prepregnancy insulin requirements (181). Insulin sensitivity then returns to prepregnancy levels over the following 1–2 weeks. For individuals taking insulin, particular attention should be directed to hypoglycemia prevention in the setting of breastfeeding and erratic sleep and eating schedules (182). Individuals with GDM usually do not require diabetes medications in the postpartum period. Contraception A major barrier to effective preconception care is the fact that many pregnancies are unplanned (183). Planning pregnancy is critical in individuals with preexisting diabetes to achieve the optimal glycemic goals necessary to prevent congenital malformations and reduce the risk of other complications. Therefore, all individuals with diabetes of childbearing potential should have family planning options reviewed at regular intervals to make sure that effective contraception is implemented and maintained. This applies to individuals in the immediate postpartum period. Individuals with diabetes have the same contraception options and recommendations as those without diabetes, although the existence of diabetes complications or other vascular disease may modify recommended

options (12). Long-acting, reversible contraception may be ideal for individuals with diabetes and childbearing potential. The risk of an unplanned pregnancy outweighs the risk of any currently available contraception option.

## # POSTPARTUM CARE

### # Recommendations

- 15.26 Insulin requirements need to be evaluated and adjusted for individuals requiring insulin after delivery because insulin resistance decreases dramatically immediately postpartum. C - 15.27 A contraceptive plan should be discussed and implemented with all people with diabetes of childbearing potential. A - 15.28 Breastfeeding efforts are recommended for all individuals with diabetes. A Breastfeeding is recommended for individuals with a history of GDM for multiple benefits, A including a reduced risk for type 2 diabetes later in life. B - 15.29 Postpartum care should include psychosocial assessment and support for self-care. E - 15.30 Screen individuals with a recent history of GDM at 4–12 weeks postpartum, using the 75-g oral glucose tolerance test and clinically appropriate nonpregnancy diagnostic criteria. B - 15.31 Individuals with a history of GDM should have lifelong screening for the development of type 2 diabetes or prediabetes every 1–3 years. B - 15.32 Individuals with overweight or obesity and a history of GDM found to have prediabetes should receive

Special Postpartum Considerations for Individuals With Gestational Diabetes Mellitus Because GDM often represents previously undiagnosed prediabetes, type 2 diabetes, maturity-onset diabetes of the young, or even developing type 1 diabetes, individuals with GDM should be tested for persistent diabetes or prediabetes at 4–12 weeks postpartum with a fasting 75-g OGTT using nonpregnancy criteria as outlined in section 2, “Diagnosis and Classification of Diabetes,” specifically Tables 2.1 and 2.2. The OGTT is recommended over A1C at 4–12 weeks postpartum, because A1C may be persistently impacted (lowered) by the increased red blood cell turnover related to pregnancy, by blood loss at delivery, or by the preceding 3-month glucose profile. The OGTT is more sensitive at detecting glucose intolerance, including both prediabetes and diabetes, and has been examined as a screening tool for these conditions in the first 12 weeks after delivery in individuals who had a recent pregnancy with GDM (188,189). OGTT testing immediately postpartum, while still hospitalized, has demonstrated improved engagement in testing but also variably reduced sensitivity to the diagnosis of impaired fasting glucose, impaired glucose tolerance, and type 2 diabetes (190,191). In a small subset of individuals who are unable to tolerate or who decline the postpartum glucose.

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tolerance test, an A1C performed at 6–12 months postpartum may be considered. A retrospective study aimed to evaluate the performance of A1C as a postpartum screening test (192). The study included 677 out of 9,118 individuals with a recent GDM diagnosis who had both A1C and 2-h 75-g OGTT testing performed for clinical purposes between delivery and 12 months postpartum. A1C level of 5.7% had 80% sensitivity and 80.8% specificity for the detection of a 2-h plasma glucose value  $\geq 200$  mg/dL ( $\geq 11.1$  mmol/L). A weakness of this study is the likelihood of selection bias, as it is not standard of care to perform both tests unless there is higher concern for a diagnosis of diabetes. Individuals with a history of GDM should have ongoing screening for prediabetes or type 2 diabetes every 1–3 years, even if results of the initial 75-g OGTT at 4–12 weeks postpartum are normal. Ongoing evaluation may be performed with any recommended glycemic test (see section 2, “Diagnosis and Classification of Diabetes”). Individuals with a history of GDM have an increased lifetime maternal risk for diabetes estimated at 50–60% (193,194), and those with GDM have a 10-fold increased risk of developing type 2 diabetes compared with those without GDM (193). Absolute risk of developing type 2 diabetes after GDM increases linearly through a person’s lifetime, being ~20% at 10 years, 30% at 20 years, 40% at 30 years, 50% at 40 years, and 60% at 50 years (194). Hazard ratios for incident diabetes were significantly elevated for a history of GDM in a single pregnancy but were even higher for a history of two GDM pregnancies in a large retrospective cohort study (hazard ratios ranged from 4.35- to 15.8-fold based on number of pregnancies with GDM and which pregnancy the individual had GDM (first or second) (195). In the prospective Nurses’ Health Study II (NHS II), subsequent diabetes risk after a history of GDM was significantly lower in those who followed healthy eating patterns (196). Adjusting for BMI moderately attenuated this association. Interpregnancy weight gain is associated with increased risk of adverse pregnancy

outcomes (197) and higher risk of GDM, while in people with BMI >25 kg/m<sup>2</sup>, weight loss is associated with lower risk of developing GDM in the subsequent pregnancy (198). Development of type 2 diabetes is 18% higher.

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